NEW YORK UROLOGY SPECIALISTS
BETTER SCIENCE. BETTER HEALTH.

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June 14, 2025

Dr. Mehmet Oz, Administrator
Attention: CMS-0042-NC
Centers for Medicare & Medicaid Services
Department of Health and Human Services
P.O. Box 8013
Baltimore, MD 21244–8013
HealthTechRFI@cms.hhs.gov .

RE: File code CMS-0042-NC

Dear Administrator Dr. Mehmet Oz,

We are submitting our comments on the proposed rule: "**Request for Information: Health Technology Ecosystem."**

As a background, New York Urology Specialists is a small, independent medical practice providing medical and surgical urological care to patients. We operate in a highly **competitive market**. We differentiate ourselves by offering cost-effective, affordable care enabled by technology and automation whenever possible. Our ability to stay competitive and offer affordable care is contingent on being productive and efficient and on having **low overhead**.

We have been advocates for Administrative Simplification and national standards. New York Urology Specialists has been active in advocating for the enforcement of HIPAA Administrative Simplification national standards, which play an essential role in our low-operating-cost business model and allow us to provide care more efficiently and at a lower cost. National standards that lower the administrative costs of healthcare are an important factor in our ability to stay price competitive, especially when many patients have high deductibles and co-insurance costs.

New York Urology Specialists in the past had been a member of WEDI, where we contributed to the development of the **WEDI** REMITTANCE ADVICE & PAYMENT SUBWORKGROUP White Paper, "Best Practices for Health Plans that Sub-Contract Creation of the Remittance Advice," dated September 13, 2022. We also submitted numerous **Briefing Papers to the CMS Office of Burden Reduction** on issues related to HIPAA Administrative Simplification requirements such as "Telecommunication Fees," "Direct vs Indirect Standard Transactions." New York Urology Specialists has filed **over 100 valid complaints** with the CMS ASETT program related to health plan non-compliance with HIPAA Administrative Simplification requirements.

General Comments: Health Technology Ecosystem.

We commend CMS for addressing the challenges of modernizing the healthcare ecosystem with up-to-date IT capabilities.

Fundamentally, the problem with the electronic exchange of information among healthcare providers is that there is **no standardized enumeration of health plans as required by the HIPAA Act (Health Plan ID),** as CMS withdrew the regulation; as a result, electronic **identification** of health plans is nearly impossible. The problem became obvious with the implementation of the No Surprises Act, as there is no clarity as to how to contact the payer electronically, who the payer is, and whether the payer is self-insured or fully insured and may thus be regulated under state-based laws. The same problem on a small scale surfaced with the implementation of the **CMS–0057–P (prior Authorization and Interoperability requirements), as there is no way to identify health plan identity and endpoints electronically.**

The healthcare ecosystem consists of more than 1 million providers, more than 88,000 **legally distinct health plans,** each with its own rules, and thousands of various "clearinghouses" and "vendors." The current system requires 1-to-1 connections between providers and health plans as well as health plan vendors. There is **no practical way to connect** billions of 1-to-1 connections for each electronic transaction. The problem became apparent with the **Change Healthcare security breach**, which required millions of providers to re-enroll with millions of health plans in electronic transactions without a standard HPID; millions of hours of manual work had to be invested to manually change proprietary Payer IDs that different clearinghouses assign to health plans. More than a year after the Change Healthcare breach, many providers are back to receiving explanation of benefits by mail and payments by check.

A national electronic transaction clearinghouse/warehouse, along with a Health Plan ID (HPID), is necessary, which we will discuss again below.

An Adequate Infrastructure And Essential Requirements To Implement Nationwide Interoperability Do Not Exist And Have Not Been Implemented By CMS.

It is not possible to implement an Electronic Exchange of Medical Information without having a fully functioning implementation of HPID (Health Plan ID)

As we wrote to ONC on 3/24/2022 in response to ePrior Authorization **RIN–0955–AA04 ""**Request for Information: Electronic Prior Authorization Standards, Implementation Specifications, and Certification Criteria,

The **first recommendation is that a standard requirement for Health Plan Identification (HPID) is implemented** nationally, similarly to how the NPI database exists for healthcare providers. Without the

ability to electronically identify each of the over 84,000 plans (and even more products and vendors), automation is **neither possible nor feasible**. If CMS, along with the industry, cannot figure out how to implement HPID, it is a hard sell for CMS to try to "pull off" any other standard in Health IT.

For a health IT vendor trying to serve thousands or millions of patients across the country belonging to one of more than 88,000 health plans, there is no feasible way to connect electronically to health plans without having an adopted national standard Health Plan ID with detailed information about it's status, contact information, digital endpoints and legal status. Before attempting a National Provider Directory, CMS/HHS should fix the existing problem, the failure to issue a HPID.

The provider or health IT vendor should only need to contact the health plan electronically and then be electronically guided on how to proceed via API using a standard health plan ID (HPID) or standard national healthcare business associate ID.

It should be the **health plan's responsibility to build and maintain real-time API connections to all its 'business associates**" and return all information through ONE back connection to the healthcare provider. **Healthcare providers or health IT vendors should NOT be in the position of running the health plan's IT operations.**

National Data-Warehouse Exchange Hub / Clearinghouse

CMS needs to take a more 'global' view of the healthcare system. It is necessary to consider how a small healthcare provider or a rural healthcare provider, or even a relatively large provider can possibly manage the complexity of "electronic" healthcare.

A health plan directory, an ERA transaction, or a prior authorization are only some of the many "administrative tasks" that health care providers have to do using electronic transactions. API connections to external patient-facing applications and health IT vendors just add to the complexity of "running a medical practice." There are many others. Maintaining connections to each "service" or 'administrative task" is a financially and administratively costly task.

There are more than **84,000 Employer health plans** according to the Department of Labor (Group Health Plans Report Abstract of 2022 Form 5500 Annual Reports Reflecting Statistical Year Filings Data Extracted on 5/8/2024 August; https://www.dol.gov/agencies/ebsa/researchers/data/group-health-plan-data). In addition, there are **more than 4,000 Medicare Advantage and Medicaid Managed care plans**.

According to the National Clearinghouse industry association, Cooperative Exchange, 8**,000 payers and 1,000 HIT vendor connections are maintained by its members.** These numbers reflect the fact that even with these large numbers, the system is not nearly interconnected as each HIT vendor does not have a connection to each health plan or even each payer. Clearly, it is not feasible for every single clearinghouse to maintain and support over time so many non-standard connections or even standard 1-to-1 connections.

The infrastructure is failing with essentially only 3 functional electronic transactions: 270/271 eligibility, and 835 ERA/EFT transactions. Adding dosens or hundreds of transactions is not feasible without fixing the underlying system.

It is not **feasible and inefficient** for healthcare providers to maintain numerous connections for various standard transactions to numerous payers and government organizations such as CMS. Adding another 'transaction' and unique connection to **these requirements is simply adding to costs and administrative burdens.** It is also not feasible for 84,000 health plans or >8,000 payers to have 1-to-1 connections for Payer-to-Payer data exchange.

To address this problem, we recommend that **a national 'data exchange hub/clearinghouse'** using standard real-time API connections that are created and maintained by the Federal Government and possibly a competing member-owned private option. This will allow providers and health plans or their clearinghouses **to maintain only one standard direct connection** through which data for all electronic transactions can seamlessly flow to any other healthcare industry participant. All that would be necessary is a transaction ID and the HPID.

Having a national data-warehouse exchange hub/clearinghouse **would vastly simplify and streamline the system and solve the inter-connectivity problem that has not been solved** for 835 ERA transactions, 11 years after the deadline for compliance, as evidenced by lack of adoption, the prevalence of paper EOBs, multitude of provider complaints to the CMS Office of Burden Reduction, division of National Standards that administers the ASETT Complaint system. This will also significantly lower the transactional costs, a major goal enshrined in HIPAA Law. It will also allow the same connection to be used to submit / update data electronically and securely for provider-to-heatlh plan, Patient Access API, and Provider Access API.

The security of electronic connections in healthcare is a major issue. These transactions carry HIPAA privacy information, valuable commercial information, and financial information. Authenticating and maintaining one connection is a much more secure way to transact electronically than trying to maintain and authenticate hundreds or thousands of point-to-point connections.

The security problem came to the forefront with the Change Healthcare security breach. With the need to maintain 1-to-1 payer to provider connections, the risks are multiplied by billions.

Example of a Functional National Data Warehouse Exchange Hub / Clearinghouse

An **example of a functional 'national' data-warehouse exchange hub/clearinghouse** is the NACHA-governed ACH system that underpins the healthcare EFT/ACH transaction. The national/international system of ACH is governed by NACHA rules. It is run via two central data-warehouses/transaction clearinghouses, the Federal Reserve Bank runs one, and a competing industry-owned clearinghouse, "The Clearing House," processes about 50% of the transactions. The two clearinghouses have interconnections governed by NACHA that allow a seamless connection

between >10,000 financial institutions nationwide with bank-level security (https://www.theclearinghouse.org/payment-systems/ach and https://www.federalreserve.gov/paymentsystems/fedach_about.htm)

This would allow each healthcare provider, each clearinghouse, each health plan, and each Health IT consumer APP or "digital tools" or Digital health products to **maintain only ONE connection** for all healthcare-related electronic transactions, including 835 ERA, prior authorization, claim attachments, NDH, patient access - a far **superior** and less costly solution than the current state of affairs. This will lower the cost of entry into the industry, increase competition in the clearinghouse industry, and promote innovation.

It is **not** feasible for health providers or their EHRs to manage thousands of vendor connections who interested in accessing the data on behalf of thousands or millions of current and past patients.

The proposed **"national' Data-Warehouse Exchange hub,** as described above, is the only practical way to address the problems identified by the ONC/CMS. Anyone "connected" to the national Data Warehouse Exchange Hub, would be deemed a "trusted" party to engage in API transactions.

API Standards

CMS proposal for API standards is a helpful step to greater interoperability. However, CMS has failed to properly account for the complexity of this implementation with >1 million health care providers and >88,000 health plans needing to create 1-to-1 APIs and secure connections for each of the proposed standards, that's potentially >88 BILLION connections (1 million plans x 88,000 health plans) for each transaction. As CMS/HHS proposes more and more electronic standards, this will become even more unwieldy. CMS should step back **and implement a National Warehouse** as described above so that each plan, each provider has to maintain only one incoming/outgoing (2-way connection).

Failed Enforcement of Administrative Simplification Requirements Cannot Support the Adoption of National Standard for Electronic Transactions, including Prior Authorization and Interoperability: 45 CFR 162.

The fundamental problem is that CMS adopted an unprecedented **"voluntary" compliance mechanism** and "informal" enforcement for **45 CFR 162** (45 CFR § 160.312). This approach has failed for every transaction adopted previously – the result is that there is rampant non-compliance; CMS's own data shows as much

Section 1172(b) of the act indicates that the standard can only be adopted with the **objective of reducing administrative costs**. As New York Urology Specialists' complaints and FOIA requests more than adequately demonstrated, the current enforcement mechanism has failed to achieve "reduced" costs to providers and in fact, led to 3-10% **increase in costs** due to virtual credit cards and EFT fees. Numerous industry organizations, such as the MGMA, AMA, and AHA have complained about this as well.

Provider-to-Provider "real-time" communication.

CMS must establish standards that allow provider-to-provider real-time digital communications similar to "chat" across different EMR platforms. CMS reimbursement policy must support this modern necessity to improve care coordination, patient outcomes and physician-to-physician communications.

2. Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

All relevant data is valuable. One problematic point with EHRs is that they do not allow patients to share the contents of their health records with providers using other EHRs. When I see a patient in the office, the patient should be able to pull up their app and share all the information they want to share with another provider in real-time, electronically. Patients should have the ability to add other providers that are not part of the healthcare organization / EHR they see to their chart and forward the data electronically seamlessly, electronically, to other providers across platforms.

PC-11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

a. How valuable, available, and accurate do you find the data they share to be?
   HIEs are not very valuable. A large number of my patients in New York go for the winter to Florida; there is no data sharing with HIEs from Florida.
   The local HIEs data sharing is hit-and-miss; the need to get consent to share data from each organization is a significant barrier as often I need to see data off-hours, when I am reviewing patient test results. In addition, the inability to import data from HIE into my EMR for my use, including to plot discrete lab data along with our results is very limiting. We choose not to use HIE as there is rarely practical value to the data.
b. **What changes would you suggest?**
   a. Nationawide HIE connections. I should be able to access data from any provider anywhere in the country.
   b. Should be able to import discrete standardized data, such as for labs from HIE to our EMR, and plot it on a timeline. Typing in data from HIE to my notes is a useless exercise. Seeing a CT report but not being able to import a copy into my chart makes it nearly worthless.

c. I routinely order and see other providers order repetitive labs because I need to know answers, and it is impossible to access and import into my EHR recent labs that the patient had in another practice. Data in PDF or scanned is useless and impairs my productivity.

PC-13. How can CMS encourage patients and caregivers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would increasing reporting of complaints advance or negatively impact data exchange?

CMS/ONC is asking the **wrong question**. For all electronic transactions, the ability to submit technical reports of malfunction or violations of the law should be **built into the API transactions**. In this case, the patient should NOT need to go to the ONC website but rather **should be able to submit a complaint from their Patient Portal/App**. Similarly, for prior authorization API, providers and patients should be able to use the same API interface to submit complaints electronically directly to the appropriate CMS office.

C. Providers

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

When it comes to IT, the issues identified previously, the lack of a **national Electronic Transaction Warehouse**, and a **lack of standard Health Plan ID** that allows full identification of the plan and electronic connectivity are two major problems in the development of Health IT and in particular in making administrative tasks (prior authorization, billing, etc) electronic.

We request a mandate that **ONLY one connection to the health plan** is mandated/allowed, the **health plan must deal with all its vendors internally**, and return results VIA ONE connection to the provider. Providers should not have to figure out who the vendors are and should not incur the **additional** costs of dealing with them directly.

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:

Ideally, there should not be faxed or scanned documents that are important.

CMS should focus on making the need for faxes/scanned documents to disappear.

The lab results should be shareable / accessible by any provider in a structured standard data format and importable into the system; the patient should be able to share data electronically in a structured format with ANY provider in the US.

Making every non-standard document interchangeable is very costly and burdensome; many are internal practice documents that will just create unnecessary work reviewing them to the receiving provider with little clinical value. Classifying every non-standard document is very costly as well for the practice. Perhaps these should be available as a category “Procedure/Surgical reports” – classifying them in great detail is practically impossible. “Labs”.

PR-4. What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment?

Again, National Electronic Transaction Warehouse is a critical part that is needed to take the healthcare system into the future. Providers should manage ONLY one connection to the outside world to the National Electronic Transaction Clearinghouse/Warehouse. It is not feasible to manage thousands of apps trying to connect to provider/EHR.

PR-14. How can CMS encourage providers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

CMS/ONC is asking the **wrong question**. For all electronic transactions, the ability to submit technical reports of malfunction or violations of the law should be **built into the API transactions**. In this case, the provider should NOT need to go to the ONC website but rather **should be able to submit a complaint from the EHR using API**. Similarly, for prior authorization API, providers and patients should be able to use the same API interface to submit complaints electronically directly to the appropriate CMS office.

CMS needs to lower the administrative barriers to filing complaints and make the ability to file complaints seamless, part of the routine use of the API interfaces.

3. Digital Identity

The question that needs to be answered is **who is going to pay for all of this?** The patient? Health plan? CMS? There should be a national standard/validation of "trust". Providers do not want to manage and wet dozens or hundreds of vendors.

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers? a. How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?

There is no interest in sharing information with payers unless and until there is actual legitimate ability by providers to negotiate contractual terms, the use of data and **analysis methodology is transparent**, and the **algorithms are open-source**, reviewable by third parties on GitHub.

As CMS considers Optum the paragon of health IT data crunching, I have yet to see a legitimate use of data by Optum. The "analytics" I have seen is biased to produce results that are financially favorable to Optum's interests. **So thank you, "NO."**

We appreciate the opportunity to provide our comments to CMS. If you have any questions, please do not hesitate to contact Dr. Alex Shteynshlyuger (dralex@newyorkurologyspecialists.com).

Sincerely,

Recoverable Signature

X Alex Shteynshlyuger

Signed by: bfc3d9f1-f174-4414-987c-b7d67d80a30d

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